

# Toremifene (Fareston®)

Toremifene is a cancer drug. It is used to treat breast cancer.

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## What is toremifene (Fareston®)?

Toremifene is a hormonal therapy drug. It is also called Fareston<sup>®</sup>. It is used to treat breast cancer. You may have it if you have been through the menopause. It can be given alone or with other types of treatment. It is best to read this information with our general information about [hormonal therapies](#) and [breast cancer](#).

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

## More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

## How toremifene works

Hormones are chemicals that our bodies make. They act as messengers and help control how cells and organs work. Hormonal therapy drugs change how hormones are made or work in the body.

Many breast cancers rely on the hormone oestrogen (or estrogen) to grow. This is called oestrogen-receptor positive (ER positive) breast cancer. Toremifene blocks oestrogen from reaching the cancer cells. This means the cancer may grow more slowly. Or it may stop growing altogether.

## How toremifene is given

You have toremifene as tablets you can take at home.

During your treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

## Taking toremifene tablets

You may have toremifene on its own, or with other drugs. Your cancer team will discuss your treatment plan with you.

You usually take toremifene once a day. You can take it with or without food. If you forget to take a tablet, take the next tablet as usual. Do not take a double dose. If you have missed more than 1 dose, tell your doctor or nurse.

Always take the tablets exactly as they tell you to. This is important to make sure they work as well as possible for you.

Other things to remember about your tablets:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Get a new prescription before you run out, and make sure you have plenty for holidays.
- Do not throw away unused tablets. Return them to a pharmacy.

Your cancer team may also give you other medicines to take home. Take all your medicines exactly as they tell you to. Do not stop taking any of your medicines unless your doctor tells you to.

## About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

When you have this treatment, you will have regular appointments with a doctor, nurse or pharmacist. Always tell them about any side effects you have. They can give you:

- drugs to help control most side effects
- advice about managing side effects.

Most side effects can be managed. But sometimes side effects are harder to control. It is important not to stop taking hormonal therapy without telling your doctor. If side effects cannot be managed, your doctor may suggest a different type of hormonal therapy.

## Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

## Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

## Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

### Hot flushes and sweats

Hot flushes are a common side effect of this treatment. During a flush, your neck and face may feel warm. If you have white skin, your skin may look red. If you have black or brown skin, your skin colour may deepen. Flushes may last up to 5 minutes. You might also have sweats or feel anxious or irritable during a hot flush.

Here are some things you can do to reduce the effects of hot flushes:

- Wear natural fabrics, such as cotton.
- Wear layers so you can remove clothes as needed.

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- Use cotton sheets and layers of bedding you can remove.
- Try cooling pads or pillows to keep you cool.
- Keep rooms cool or use a fan.
- Have cold drinks and avoid caffeine and alcohol.

You might have fewer hot flushes as your body adjusts to this treatment. You could try therapies to help you [cope with hot flushes](#), such as:

- [talking therapies](#) like cognitive behaviour therapy (CBT)
- controlled breathing exercises
- yoga
- acupuncture.

If they do not improve, talk to your doctor. Certain drugs can help to improve hot flushes.

Flushes usually stop a few months after treatment ends. But some people continue to have them.

## Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

### Vaginal discharge

You may have a white or yellowish vaginal discharge when taking this treatment. Or you may bleed from the vagina. If bleeding continues for more than a few days, tell your doctor or nurse.

### Feeling sick

You may [feel sick](#) at times when you start taking toremifene. This often lessens or goes away after a few weeks.

### Fluid build-up

This treatment can cause a build-up of fluid in the body. This will slowly get better after treatment ends. Contact the hospital on the 24-hour number if you:

- are gaining weight
- have swelling in your face, legs or ankles.

They can give you advice and treatment to help.

### Feeling tired (fatigue)

[Feeling tired](#) is a common side effect. Try to pace yourself and plan your day so you have time to rest between activities.

Being physically active can help to manage tiredness and give you more energy. It also:

- helps you sleep better
- reduces stress
- improves your bone health.

If you feel sleepy, do not drive or operate machinery.

## Skin changes

This treatment can [affect your skin](#). It may cause a rash, which may be itchy. Your doctor or nurse can tell you what to expect. If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day.

Always tell your doctor or nurse about any skin changes. They can give you advice and may prescribe creams or medicines to help. Changes to your skin usually improve when treatment ends.

## Dizziness or changes in vision

This treatment may make you feel dizzy. Rarely, it causes blurred vision or changes in eyesight. Always tell your doctor or nurse if you have any of these side effects. If you feel dizzy or have changes in your sight, do not drive or operate machinery.

## Mood changes

You may feel low or depressed, or have mood swings, during this treatment. Talking to family and friends about how you feel might help. If mood changes last for more than a few weeks, tell your doctor, nurse or pharmacist. They can talk to you about different ways to manage low mood or [depression](#).

## Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

## Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

## Increased risk of womb cancer

If you have been through menopause and take toremifene over a long period of time, you may have a slightly increased risk of [cancer of the womb](#). The benefits of taking toremifene outweigh this risk. Your doctor may arrange for you to have tests each year to check your womb. If tests do find womb cancer, early treatment is very successful.

If you have vaginal bleeding after the menopause, tell your doctor. They can check for the cause. It can be an early sign of womb cancer, but it is usually caused by conditions other than cancer.

## Effects on the heart

This treatment might increase certain risk factors for [heart problems](#). Tell your doctor if you have:

- had any problems with your heart
- high blood pressure
- high levels of cholesterol – a fatty substance in your blood.

They can monitor or treat these if needed. Talk to your doctor if you are worried about this.

Things you can do to take care of your heart include:

- not smoking
- maintaining a healthy weight
- being physically active.

## Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

## Difficulty sleeping (insomnia)

Sometimes this treatment can affect your sleep. Tell your doctor, nurse or pharmacist if cancer treatment makes it [difficult to sleep](#).

## Other information about

### Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online

- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

## Lactose

This treatment may contain lactose. If you have a lactose allergy or intolerance, ask your doctor, nurse or pharmacist for more information.

## Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

## About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

## References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

## Date reviewed

Reviewed: 01 June 2025 | Next review: 01 June 2027

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

## The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

## How we can help

### How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

### Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



#### Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



#### Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

#### Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



#### Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



## Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

**Get help**